Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0005	**Age / Sex**	57 / M
Setting	Emergency Dept	**Primary dx**	Severe hypoglycemia, insulin-treated T2DM

Chief complaint

Found unresponsive and sweating by his wife.

History of present illness

Skipped lunch after morning gardening. Evening: wife found him drowsy, then unrousable. Home capillary glucose reading 1.8 mmol/L. Last insulin dose (glargine plus aspart) taken at 13:00.

Past history / medications

Type 2 diabetes for 15 years on basal-bolus insulin, chronic kidney disease stage 3, hypertension.

Examination

Diaphoretic, HR 108, BP 142/84, temp 36.2 C. Pupils equal, reactive. No lateralizing signs, brief twitching of the right hand. Tone normal.

Glasgow Coma Scale

Eyes open only to pain. Utters isolated inappropriate words. Motor: withdraws from painful stimulus.

GCS on arrival	Score	Response
Eye opening (E)	2	To pain
Verbal (V)	3	3 - Inappropriate words
Motor (M)	4	Withdraws from pain
TOTAL GCS		**9**

GCS 15 min after IV dextrose	Score	Response
Eye opening (E)	4	Spontaneous
Verbal (V)	5	5 - Oriented
Motor (M)	6	Obeys commands
TOTAL GCS		**15**

Investigations

Venous glucose 1.6 mmol/L. Creatinine 142 umol/L. Electrolytes normal. ECG sinus tachycardia. CT head not required after full recovery.

Assessment

Hypoglycemic coma from insulin and missed meal, reduced insulin clearance in CKD. Initial GCS 9.

Plan / disposition

IV dextrose 10% bolus 150 mL with prompt recovery to GCS 15 within 15 min, then infusion and oral carbohydrate. Observed 12 h for recurrent hypoglycemia (long-acting insulin). Diabetes team to adjust insulin; hypoglycemia education.